

EMA REGULATORY UPDATE · FEBRUARY 2026

GVP Population-Specific Considerations III

Pregnant & Breastfeeding Women

What every pharmacovigilance professional needs to know about the new EMA GVP guideline.

WHY THIS GUIDELINE MATTERS

A long-standing gap in pharmacovigilance.

Pregnant and breastfeeding women are **historically underrepresented** in clinical trials — yet they represent a significant proportion of real-world medicine users.

THE CHALLENGE

Lack of standardised PV requirements for this population leaves safety data incomplete and compliance obligations unclear.

THE SOLUTION

GVP P III sets dedicated, legally enforceable requirements — closing the regulatory gap for the first time.

SCOPE OF GVP P III

Three populations. One comprehensive guideline.

1

Pregnant Women

Medicines taken during any trimester of pregnancy

2

Breastfeeding Women

Medicines used during the breastfeeding period

3

Children Exposed In Utero or via Breastmilk

Neonates and infants with indirect medicine exposure

Key Requirement 1 of 4



ICSR COLLECTION & FOLLOW-UP

Individual Case Safety Reports in pregnancy

- Structured collection of ICSRs from pregnant and breastfeeding patients, including **systematic follow-up for pregnancy outcomes**
- Mandatory documentation of neonatal outcomes, including birth defects, prematurity, and infant adverse events via breastmilk
- Follow-up timelines and minimum data set requirements for pregnancy reports aligned with EU E2B(R3) standards

Action: Review and update your ICSR follow-up SOPs to include pregnancy-specific data fields and outcome documentation requirements.

Key Requirement 2 of 4



RISK MANAGEMENT PLANS

RMP updates for reproductive safety

- **Pregnancy prevention programmes (PPPs)** must be reviewed against the updated GVP P III and Module XVI Rev. 3 requirements
- RMP Section C (Risk Minimisation) must address **breastfeeding-specific risk communication** strategies
- Additional Risk Minimisation Measures (aRMMs) must be proportionate and include **measurable effectiveness KPIs**

Note: Module XVI Addendum I (effective Aug 2025) provides supplementary guidance on risk minimisation for medicines with embryo-fetal toxicity risks. Review alongside GVP P III.

Key Requirement 3 of 4



PSUR / PBRER REPORTING

Benefit-risk assessments for special populations

- PSURs/PBRERs must now include **dedicated population-specific benefit-risk sections** covering pregnant and breastfeeding women
- Aggregate analysis of cumulative pregnancy exposure data required, including outcomes from registries and PASS studies
- Real-world data from pregnancy registries and cohort studies must be **systematically evaluated** within the PBRER framework

Action: Update your PSUR/PBRER templates now. The February 2026 effective date means these changes apply to the next reporting cycle.

Key Requirement 4 of 4



SIGNAL DETECTION

Targeted reproductive safety signal detection

- Signal detection activities must **specifically target reproductive and developmental toxicity signals** including teratogenicity and embryo-fetal toxicity
- MAHs must evaluate pregnancy-related signals both within their internal safety database **and via EudraVigilance monitoring**
- Signal management SOPs must be updated to include **population-specific search strategies** and disproportionality analyses for this population

MAH ACTION CHECKLIST

6 steps to implement GVP P III.

1

Conduct a **gap analysis** of your PV system against GVP P III requirements

2

Update **ICSR SOPs** for pregnancy report collection, follow-up and outcome documentation

3

Revise your **Risk Management Plan** Section C for pregnancy/breastfeeding risk minimisation

4

Update **PSUR/PBRER templates** to include population-specific benefit-risk assessments

5

Evaluate whether a **pregnancy registry or PASS** is required or appropriate for your product

6

Train PV and regulatory teams on updated requirements and review SmPC/PIL for compliance



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